

D119

Pediatric Primary Care for the Advanced Practice Nurse

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
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Items

1.

A 20-month-old comes in for a sick visit for a mild cough. His family recently moved from another state and his older sister was an early talker. His mother mentions he used to say about ten words but over the last two months has stopped using them and no longer points to show her things. What is the best next step?

- A. Complete a standardized developmental and autism screen and refer for evaluation and early intervention.
- B. Attribute the change to the stress of the move and recheck in six months.
- C. Reassure the mother that boys often talk later and recheck at the 2-year visit.
- D. Order a hearing test only and wait for results before any other steps.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2.

A 4-year-old is behind on vaccines and is seen for a catch-up visit. She has a runny nose and a temperature of 37.9 °C, and she is on day 5 of amoxicillin for an ear infection. Her father says his cousin had a bad reaction to a vaccine years ago. What is the best plan for today's vaccines?

- A. Wait until the antibiotic course is finished, then vaccinate.
- B. Give the catch-up vaccines today.
- C. Defer all vaccines because of the family history of a reaction.
- D. Never vaccinate when any fever is present.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3. [TRANSFER — reversed stem / different variable]

Which finding is a true contraindication to giving a child the next dose of a vaccine?

- A. A sibling who had a febrile seizure after vaccination.
- B. Anaphylaxis after the previous dose of that same vaccine.
- C. Redness and soreness at the site after the previous dose.
- D. A current ear infection being treated with antibiotics.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A 3-year-old weighs 18 kg today; she weighed 16 kg at her visit last year. She is diagnosed with acute otitis media and will receive high-dose amoxicillin at 90 mg/kg/day divided twice daily. Her older brother had the same infection last winter. What is each dose?

- A. 810 mg
- B. 1,620 mg
- C. 540 mg
- D. 720 mg

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

5. [TRANSFER — reversed stem / different variable]

A child is prescribed 600 mg of amoxicillin twice daily, which equals 80 mg/kg/day. How much does the child weigh?

- A. 7.5 kg
- B. 15 kg
- C. 30 kg
- D. 13.3 kg

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

6.

A 3-year-old who attends a busy daycare center is seen for follow-up three weeks after treatment for an ear infection. He has no fever or ear pain and is eating well. His mother is a teacher and asks many questions. Otoscopy shows an amber fluid level behind a gray, non-bulging tympanic membrane with reduced mobility. What is the most appropriate plan?

- A. Recommend he stay home from daycare until the fluid clears.
- B. Start a second course of amoxicillin for recurrent acute otitis media.
- C. Refer immediately for ear tubes.
- D. Watchful waiting for otitis media with effusion, with recheck and hearing assessment if it persists.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

7.

At a well visit, a 9-year-old who plays on a traveling soccer team has a BMI at the 91st percentile for age and sex. Her height is at the 75th percentile and she has grown 6 cm since last year. How should her weight status be classified?

- A. Overweight.
- B. Healthy weight, because she is active in sports.
- C. Healthy weight, because her height is at the 75th percentile.
- D. Obesity.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

8. [TRANSFER — reversed stem / different variable]

A 6-year-old is classified as having obesity on the BMI-for-age chart. What BMI percentile range must he fall in?

- A. A BMI of 30 or higher.
- B. Above the 99th percentile only.
- C. At or above the 85th percentile.
- D. At or above the 95th percentile.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

9.

A 10-year-old with persistent asthma on a daily low-dose inhaled corticosteroid comes in for a routine check. He is breathing comfortably today and his lungs are clear. His family just adopted a puppy. He reports using his albuterol inhaler 4 days a week and waking at night with cough twice a month. What is the best next step?

- A. Stop the inhaled corticosteroid and use albuterol only as needed.
- B. Continue the current plan, since his lungs are clear today.
- C. Check inhaler technique and adherence, then step up controller therapy.
- D. Classify asthma as well controlled because night waking is only twice a month.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

10. [TRANSFER — reversed stem / different variable]

Which history best fits well-controlled asthma in a school-age child?

- A.** Needs no albuterol but skips gym class to avoid symptoms.
- B.** Uses albuterol daily but never wakes at night.
- C.** Uses albuterol 1 day a week, wakes at night once a month or less, and plays sports fully.
- D.** Uses albuterol 3 days a week before soccer, with no night symptoms.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A 16-day-old is brought in because she felt warm. The baby was born at term after an uncomplicated pregnancy, is breastfeeding well, and looks alert and pink. Her grandmother, a retired nurse, suggests giving acetaminophen. A rectal temperature is 38.2 °C. What is the most appropriate action?

- A.** Give acetaminophen and recheck in 24 hours if the fever persists.
- B.** Send home because she is feeding well and looks well.
- C.** Order a urine test in the office and treat only if it is positive.
- D.** Arrange immediate transfer for a full sepsis evaluation and admission.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12.

A 15-year-old comes in for a sports physical with his mother. He is on the honor roll and his mother mentions he has a new part-time job. After his mother steps out, he asks if anything he says will be shared with his parents. What is the best response?

- A.** Promise that nothing he says will ever be shared with anyone.
- B.** Skip the private interview because he is doing well in school and working.
- C.** Explain that what he shares stays private unless there is a risk of serious harm to himself or others, or abuse.
- D.** Tell him his parents have a right to all information because he is a minor.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer A Concept 1

A 20-month-old comes in for a sick visit for a mild cough. ~~His family recently moved from another state and his older sister was an early talker.~~ His mother mentions he used to say about ten words but over the last two months has stopped using them and no longer points to show her things. What is the best next step?

Target word: response to a milestone concern **Decides:** Loss of previously acquired words and gestures is regression.

A	Correct	Regression of language or social skills at any age is a red flag that needs screening and referral now.
B	Camouflage	Answers the moving detail; a move does not explain losing words and pointing.
C	Absolute language	Generalizes about boys and ignores that he lost skills he already had.
D	Dropped element	Hearing testing is part of the workup, but it does not replace developmental screening and referral.

2. Answer B Concept 2

A 4-year-old is behind on vaccines and is seen for a catch-up visit. She has a runny nose and a temperature of 37.9 °C, and she is on day 5 of amoxicillin for an ear infection. ~~Her father says his cousin had a bad reaction to a vaccine years ago.~~ What is the best plan for today's vaccines?

Target word: whether to vaccinate today **Decides:** Mild illness, low-grade fever, and antibiotics are not contraindications.

A	Adjacent category	Antibiotics are a false contraindication for routine vaccines.
B	Correct	None of the listed facts is a true contraindication, and delaying leaves her unprotected longer.
C	Camouflage	A relative's reaction is not a contraindication for this child.
D	Absolute language	Mild illness with low-grade fever is not a reason to defer; moderate or severe illness is a precaution.

3. Answer B Concept 2 • transfer

Which finding is a true contraindication to giving a child the next dose of a vaccine?

Target word: identify the true contraindication **Decides:** Anaphylaxis to a prior dose or component.

A	Dropped element	Family history of reactions does not apply to this child.
B	Correct	A severe allergic reaction to a prior dose or component is a contraindication.
C	Confusion set	A local reaction is common and expected, not a contraindication.
D	Adjacent category	Mild illness and antibiotic use are false contraindications.

4. Answer A Concept 3

A 3-year-old weighs 18 kg today; ~~she weighed 16 kg at her visit last year.~~ She is diagnosed with acute otitis media and will receive high-dose amoxicillin at 90 mg/kg/day divided twice daily. ~~Her older brother had the same infection last winter.~~ What is each dose?

Target word: mg per dose **Decides:** $18 \text{ kg} \times 90 \text{ mg/kg/day} = 1,620 \text{ mg/day}$, divided into 2 doses.

A	Correct	$18 \times 90 = 1,620 \text{ mg per day}$; $1,620 / 2 = 810 \text{ mg per dose}$.
B	Dropped element	The total daily amount, not divided into two doses.
C	Adjacent category	Divides the daily dose by three, as for every-8-hour dosing, not twice daily.
D	Wrong stem number	Uses last year's weight of 16 kg.

5. Answer B Concept 3 • transfer

A child is prescribed 600 mg of amoxicillin twice daily, which equals 80 mg/kg/day. How much does the child weigh?

Target word: solve for weight **Decides:** Daily dose = 1,200 mg; weight = $1,200 / 80$.

A	Dropped element	Uses a single 600 mg dose instead of the daily total.
B	Correct	$600 \times 2 = 1,200 \text{ mg/day}$; $1,200 / 80 = 15 \text{ kg}$.
C	Inverted operation	Doubles the daily total again instead of dividing it by the mg/kg dose.
D	Confusion set	Divides by 90, the top of the high-dose range, instead of the 80 given.

6. Answer D Concept 4

A 3-year-old ~~who attends a busy daycare center~~ is seen for follow-up three weeks after treatment for an ear infection. He has no fever or ear pain and is eating well. ~~His mother is a teacher and asks many questions.~~ Otoscopy shows an amber fluid level behind a gray, non-bulging tympanic membrane with reduced mobility. What is the most appropriate plan?

Target word: diagnosis and management **Decides:** Fluid without bulging or acute symptoms = otitis media with effusion.

A	Camouflage	Answers the daycare detail; effusion is not contagious and does not require exclusion.
B	Confusion set	Treats effusion as AOM; there is no bulging or acute symptom.
C	Absolute language	Tubes are considered for persistent effusion (about 3 months or more) with hearing loss or other risk factors, not at three weeks.
D	Correct	Effusion after AOM is common, usually resolves, and is not treated with antibiotics.

7. Answer A Concept 5

At a well visit, a 9-year-old ~~who plays on a traveling soccer team~~ has a BMI at the 91st percentile for age and sex. Her height is at the 75th percentile and ~~she has grown 6 cm since last year.~~ How should her weight status be classified?

Target word: weight category **Decides:** BMI-for-age between the 85th and 95th percentile.

A	Correct	The 85th to below 95th percentile is overweight for children 2 and older.
B	Camouflage	Activity level does not change the BMI-percentile category.
C	Wrong stem number	Height percentile is not the BMI percentile.
D	Adjacent category	Obesity starts at the 95th percentile.

8. Answer D Concept 5 • transfer

A 6-year-old is classified as having obesity on the BMI-for-age chart. What BMI percentile range must he fall in?

Target word: percentile from category **Decides:** Obesity = at or above the 95th percentile.

A	Confusion set	Adult cutoff; children are classified by percentile.
B	Inverted operation	Severe obesity is defined higher, but obesity itself starts at the 95th.
C	Adjacent category	That threshold begins the overweight range.
D	Correct	That is the pediatric obesity cutoff for ages 2 and up.

9. Answer C Concept 6

A 10-year-old with persistent asthma on a daily low-dose inhaled corticosteroid comes in for a routine check. He is breathing comfortably today and his lungs are clear. ~~His family just adopted a puppy.~~ He reports using his albuterol inhaler 4 days a week and waking at night with cough twice a month. What is the best next step?

Target word: control level and action **Decides:** Rescue use more than 2 days a week = not well controlled.

A	Inverted operation	Steps down when symptoms show he needs more control.
B	Camouflage	A normal exam today does not reflect control over the past weeks.
C	Correct	Rescue use over 2 days a week means not well controlled; confirm technique and adherence before stepping up.
D	Dropped element	Ignores rescue use, which alone makes it not well controlled.

10. Answer C Concept 6 • transfer

Which history best fits well-controlled asthma in a school-age child?

Target word: pick the well-controlled pattern **Decides:** Rescue use 2 days a week or less, night waking once a month or less, no activity limits.

A	Adjacent category	Activity limitation is an impairment sign of poor control.
B	Dropped element	Daily rescue use alone means poor control.
C	Correct	Meets well-controlled thresholds in each area.
D	Wrong stem number	Rescue use over 2 days a week is not well controlled.

11. Answer D Concept 7

A 16-day-old is brought in because she felt warm. The baby was born at term after an uncomplicated pregnancy, is breastfeeding well, and looks alert and pink. ~~Her grandmother, a retired nurse, suggests giving acetaminophen.~~ A rectal temperature is 38.2 °C. What is the most appropriate action?

Target word: disposition of a febrile neonate **Decides:** Age under 28 days with a rectal temperature of 38.0 °C or higher.

A	Camouflage	Acts on the grandmother's suggestion; antipyretics can mask a serious infection.
B	Dropped element	Well appearance does not rule out serious bacterial infection in neonates.
C	Adjacent category	A partial workup suits some older infants, not a febrile newborn.
D	Correct	Fever in a neonate needs urgent full evaluation regardless of appearance.

12. Answer C Concept 8

A 15-year-old comes in for a sports physical with his mother. ~~He is on the honor roll and his mother mentions he has a new part time job.~~ After his mother steps out, he asks if anything he says will be shared with his parents. What is the best response?

Target word: how to explain confidentiality **Decides:** Confidentiality with stated limits.

A	Absolute language	Safety concerns and abuse are exceptions; an absolute promise may have to be broken.
B	Camouflage	Grades and a job do not rule out risk; every teen gets private time.
C	Correct	Honest confidentiality with its limits builds trust and supports disclosure.
D	Inverted operation	Reverses the standard of confidential adolescent care.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	7	
Wrong stem number	A figure copied from the prompt that is not the answer.	3	
Adjacent category	The right idea in the wrong bucket.	7	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	4	
Absolute language	Always, never, guarantee — where judgment applies.	4	
Confusion set	Every option is real; the test is exact role-matching.	4	
Camouflage	Answers a stem fact that was never the question.	7	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Developmental red flags	Loss of any previously gained skill (regression) at any age, or missing a key milestone window (e.g., no words by 16 months, no two-word phrases by 24 months), calls for formal screening and referral, not reassurance.	Telling parents every child develops at their own pace and waiting for the next visit.	A child who used to do something and no longer does, or a milestone clearly past its window.
2	True vs. false vaccine contraindications	Severe allergic reaction to a prior dose or a vaccine component is a contraindication; mild illness, low-grade fever, current antibiotics, and a family history of reactions are not.	Deferring vaccines for a cold or an antibiotic course.	A catch-up visit plus a minor complaint the parent mentions.
3	Weight-based dosing	Pediatric doses are ordered in mg/kg/day; multiply by the current weight, then divide by the number of doses per day.	Giving the whole daily amount as one dose, or using an old weight.	mg/kg/day plus a dosing frequency (twice daily, every 8 hours).
4	Acute otitis media vs. effusion	Acute otitis media requires a bulging tympanic membrane (or new otorrhea) with acute symptoms; fluid behind an intact, non-bulging membrane without acute signs is otitis media with effusion, which is not treated with antibiotics.	Prescribing antibiotics for any fluid or a red eardrum.	Exam findings describe fluid or redness but not bulging.
5	Growth chart and BMI categories	From age 2, BMI-for-age percentile classifies weight: 85th to below 95th = overweight; 95th or above = obesity; WHO charts are used under age 2.	Using adult BMI cutoffs or raw weight percentile for children.	A BMI percentile given for a child 2 or older.
6	Asthma control and stepping up	Rescue inhaler use more than 2 days a week, night waking, or activity limits mean asthma is not well controlled; check technique and adherence, then step up therapy.	Judging control by how the child looks in the office today.	Counts of rescue inhaler use or night waking per week or month.
7	Fever in the young infant	A rectal temperature of 38.0 °C or higher in an infant under about 28 days old calls for urgent full sepsis evaluation in a hospital setting, regardless of how well the baby looks.	Managing a well-appearing febrile newborn at home with antipyretics.	An infant's age in days plus a measured fever.

#	Concept	Rule	Trap	Tell
8	Adolescent confidentiality	Adolescents get private interview time (e.g., a HEADSS-style psychosocial history) with confidentiality explained up front, including its limits: risk of serious harm to self or others, and abuse.	Promising total confidentiality, or interviewing only with the parent present.	A teen visit where a parent is in the room or a sensitive disclosure is made.

Course scope (public description): Pediatric Primary Care for the Advanced Practice Nurse prepares the graduate nursing student to perform the role of an advanced practice nurse with the essential competencies and skills necessary to provide primary healthcare to pediatric patients, from infancy through adolescence, and their families in an outpatient setting. This course will prepare the graduate nursing student to demonstrate competence in leading health promotion and disease prevention activities; in diagnosing, managing, and coordinating care for pediatric patients with acute and chronic conditions; and in empowering patients and their families in pursuing positive health outcomes. This course will also prepare the graduate nursing student to collaborate with pediatric patients and their families in developing effective plans of care that build patient and family self-efficacy in the process of preventing and treating disease. There are no prerequisites for this course.